The Measurement of Experienced Burnout

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2 authors:

Christina Maslach
University of California, Berkeley

Susan E. Jackson
Rutgers, The State University of New Jersey

THE MEASUREMENT OF EXPERIENCED BURNOUT[1]

Christina Maslach and Susan E. Jackson[2]

University of California, Berkeley

Summary

A scale designed to assess various aspects of the burnout syndrome was administered to a wide range of human services professionals. Three subscales emerged from the data analysis: emotional exhaustion, depersonalization, and personal accomplishment. Various psychometric analyses showed that the scale has both high reliability and validity as a measure of burnout.

INTRODUCTION

The professional staff in human service institutions are often required to spend considerable time in intense involvement with other people. Frequently, the staff-client interaction is centred around the client's current problems (psychological, social, and/or physical) and is therefore charged with feelings of anger, embarrassment, fear or despair. Solutions for these problems are not always obvious and easily obtained, thus adding ambiguity and frustration to the situation. For the helping professional who works continuously with people under such circumstances, the chronic stress can be emotionally draining and poses the risk of burnout.

Burnout is a syndrome of emotional exhaustion and cynicism that occurs frequently among individuals who do "people-work" of some kind. A key

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Since the publication of this article in 1981, more extensive research was done on the MBI, which resulted in some modifications of the original measure. The present article has been re-edited to reflect those modifications. However, it does not include other new additions, including occupational norms and a more extensive bibliography of research using the MBI. This other material is contained in the MBI Manual distributed by the publisher.

[2]Now at New York University.

aspect of the burnout syndrome is increased feelings of emotional exhaustion. As their emotional resources are depleted, workers feel they are no longer able to give of themselves at a psychological level. Another aspect is the development of negative, cynical attitudes and feelings about one's clients. Such negative reactions to clients may be linked to the experience of emotional exhaustion, i.e., these two aspects of burnout appear to be somewhat related. This callous or even dehumanized perception of others can lead staff to view their clients as somehow deserving of their troubles (Ryan, 1971), and the prevalence among human service professionals of this negative attitude toward clients has been well documented (Wills, 1978). A third aspect of the burnout syndrome is the tendency to evaluate oneself negatively, particularly with regard to one's work with clients. Workers feel unhappy about themselves and dissatisfied with their accomplishments on the job.

The consequences of burnout are potentially very serious for the staff, the clients, and the larger institutions in which they interact. Our initial research on this syndrome (Maslach, 1976, 1978a, 1978b, 1979, 1982; Maslach and Jackson, 1978, 1979, 1982; Jackson and Maslach, 1982; Maslach and Pines, 1977; Pines and Maslach, 1978, 1980) along with the work of Freudenberger (1974, 1975) suggests that burnout can lead to a deterioration in the quality of care or service that is provided by the staff. It appears to be a factor in job turnover, absenteeism, and low morale. Furthermore, burnout seems to be correlated with various self-reported indices of personal distress, including physical exhaustion, insomnia, increased use of alcohol and drugs, and marital and family problems.

The initial research in this area was very exploratory, relying heavily on interviews, questionnaire surveys, and observations. Various stressors in the work environment, such as workload and ambiguity, were related to burnout, and some of these appeared to interact with individual ego level and personality characteristics (Gann, 1979). The generally consistent pattern of findings that emerged from these studies led us to postulate a specific syndrome of burnout and to devise an instrument to assess it. This measure contains three subscales tapping the different aspects of experienced burnout and has been found to be reliable, valid, and easy to administer.

CONSTRUCTION OF THE MASLACH BURNOUT INVENTORY

The items for the Maslach Burnout Inventory (MBI) were designed to measure hypothesized aspects of the burnout syndrome. The interview and questionnaire data collected during our earlier, exploratory research were a valuable source of ideas about the attitudes and feelings that characterized a burned-out worker. In addition, numerous established scales were reviewed for useful content material, although no items were borrowed outright. The general form of "recipients" was used in the items to refer to the particular people for whom the subject provided service, care or treatment. Items were written in the forms of statements about personal feelings or attitudes. The frequency that the respondent experiences these feelings was assessed using a six-point, fully anchored response format: 0 = Never; 1 = A few times a year; 2 = Once a month or less; 3 = A few times a month; 4 = Once a week; 5 = A few times a week; 6 = Every day.[3]

A preliminary form of the MBI, which consisted of 47 items with this format, was administered to a sample of 605 people (56 per cent male, 44 per cent female) from a variety of health and service occupations including: police, counsellors, teachers, nurses, social workers, psychiatrists, psychologists, attorneys, physicians, and agency administrators.[4] The resulting data were subjected to a factor analysis using principal factoring with iteration and an orthogonal (varimax) rotation. Ten factors emerged, of which four accounted for over three-fourths of the variance. A set of selection criteria was then applied to the items, yielding a reduction in the number of items from 47 to 25. Items were retained that met all of the following criteria: a factor loading greater than 0.40 on only one of the four factors, a large range of subject response, a relatively low percentage of subjects checking the 'never' response, and a high item-total correlation.

In order to obtain confirmatory data for the pattern of factors, the 25-item form was administered to a new sample of 420 people (69 per cent females, 31 per cent males) in the following occupations: nurses, teachers, social workers, probation officers, counsellors, mental health workers, and agency administrators. The results of the factor analysis on this second set of data were very similar to those of the first, and so the two samples were combined (n = 1025) for the final factor analysis.

The final factor analysis, based on the combined samples (n = 1025) and using principal factoring with iteration plus an orthogonal rotation, yielded a four-factor solution. Three of these factors had eigenvalues greater than unity and are considered subscales of the MBI (see Table 1 for the factor loadings of the final 22 items). This three-factor structure has been replicated by a number of independent researchers, including Iwanicki and Schwab (1981); Belcastro, Gold and Hays (1983); Golembiewski, Munzenrider and Carter (1983); Gold (1984); and Green and Walkey (1988).

The 9 items in the Emotional Exhaustion subscale describe feelings of being emotionally overextended and exhausted by one's work. On this factor, the item with the highest factor loading (0.84) is the one referring directly to burnout, "I feel burned out from my work." The 5 items in the Depersonalization subscale describe an unfeeling and impersonal response towards recipients of one's care or service. For both the Emotional

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[3]In addition to frequency, the original form of the MBI contained a second response dimension of intensity. However, there is now sufficient evidence that shows fairly high correlations between the two dimensions when subscale scores are computed. Therefore, the final version of the MBI only assesses frequency, and so only the relevant frequency data are presented in this revised article.

[4]The occupations represented in both this and the second sample are ones that have a high potential for burnout according to previous research (Maslach, 1976, 1978b). In all of them, the worker must deal directly with people about issues that either are, or could be, problematic. Consequently, strong emotional feelings are likely to be present in the work setting, and it is this sort of chronic emotional stress that is believed to induce burnout.

Table 1. Item factor loadings for the Maslach Burnout Inventory

	I	II	III
I. *Emotional Exhaustion*			
I feel emotionally drained from my work	0.74	0.06	0.02
I feel used up at the end of the workday	0.73	0.04	0.03
I feel fatigued when I get up in the morning and have to face another day on the job	0.66	0.18	0.15
Working with people all day is really a strain for me	0.61	0.22	-0.10
I feel burned out from my work	0.84	0.19	-0.09
I feel frustrated by my job	0.65	0.23	-0.12
I feel I'm working too hard on my job	0.56	0.08	0.07
Working with people directly puts too much stress on me	0.54	0.31	-0.06
I feel like I'm at the end of my rope	0.65	0.21	0.08
II. *Depersonalization*			
I feel I treat some recipients as if they were impersonal "objects"	0.11	0.67	-0.09
I've become more callous toward people since I took this job	0.23	0.66	-0.13
I worry that this job is hardening me emotionally	0.37	0.55	-0.10
I don't really care what happens to some recipients	0.12	0.62	-0.16
I feel recipients blame me for some of their problems	0.13	0.41	-0.04
III. *Personal Accomplishment*			
I can easily understand how my recipients feel about things	0.11	-0.06	0.50
I deal very effectively with the problems of my recipients	-0.01	-0.07	0.54
I feel I'm positively influencing other people's lives through my work	-0.02	-0.17	0.58
I feel very energetic	-0.30	-0.04	0.43
I can easily create a relaxed atmosphere with my recipients	-0.06	0.08	0.51
I feel exhilarated after working closely with my recipients	0.00	-0.23	0.55
I have accomplished many worthwhile things in this job	-0.10	-0.17	0.57
In my work, I deal with emotional problems very calmly	-0.07	0.07	0.59

Exhaustion and Depersonalization subscales, higher mean scores correspond to higher degrees of experienced burnout. Since some of the component items on each subscale had low loadings on the other, there is a moderate correlation between the two subscales (0.44). Such a correlation is in accord with theoretical expectations that these are separate, but related, aspects of burnout.

The subscale of Personal Accomplishment contains 8 items that describe feelings of competence and successful achievement in one's work with people. In contrast to the other two subscales, lower mean scores on this subscale correspond to higher degrees of experienced burnout. It isimportant to note that the Personal Accomplishment subscale is independent of the other subscales and that its component items do not load negatively on them. In other words, Personal Accomplishment cannot be assumed to be the opposite of Emotional Exhaustion and/or Depersonalization. Indeed, the correlations between the Personal Accomplishment subscale and the others are quite low: for Emotional Exhaustion it is - 0.17 and for Depersonalization it is -0.28.

Burnout is conceptualized as a continuous variable, ranging from low to moderate to high degrees of experienced feeling. It is not viewed as a dichotomous variable, which is either present or absent. A high degree of burnout is reflected in high scores on the Emotional Exhaustion and Depersonalization subscales and in low scores on the Personal Accomplishment subscale. An average degree of burnout is reflected in average scores on the three subscales. A low degree of burnout is reflected in low scores on the Emotional Exhaustion and Depersonalization subscales and in high scores on the Personal Accomplishment subscale.[5] For each subscale, the total score of the component items is computed. Given our limited knowledge about the relationship between these three aspects of burnout, the scores for each subscale are considered separately and are *not* combined into a single, overall score. Table 2 presents the subscale means and standard deviations [based on the total normative sample reported in the MBI Manual (1986), n = 11,067].

RELIABILITY

Reliability coefficients for the MBI were based on samples that were not used in the item selections to avoid any improper inflation of the reliability estimates. Internal consistency was estimated by Cronbach's coefficient alpha (n = 1,316). The reliability coefficients for the subscales were 0.90 for Emotional Exhaustion; 0.79 for Depersonalization; and .71 for Personal Accomplishment. The standard error of measurement for each subscale is as follows: 3.80 for Emotional Exhaustion; 3.16 for Depersonalization; and 3.73 for Personal Accomplishment.

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[5]At present, scores are considered high if they are in the upper third of the normative distribution, average if they are in the middle third, and low if they are in the lower third. These normative distributions and the numerical cut-off points for different occupational subgroups are contained in the MBI Manual (1986) published by Consulting Psychologists Press.

Table 2. Means and standard deviations for the subscales of the MBI

	MBI subscales		
	Emotional Exhaustion	Depersonalization	Personal Accomplishment
M	20.99	8.73	34.58
SD	10.75	5.89	7.11

n = 11,067

Data on test-retest reliability of the MBI were obtained from a sample of graduate students in social welfare and administrators in a health agency (n = 53). The two test sessions were separated by an interval of 2-4 weeks. The test-retest reliability coefficients for the subscales were 0.82 for Emotional Exhaustion; 0.80 for Personal Accomplishment; and 0.60 for Depersonalization. Although these coefficients range from low to moderately high, all are significant beyond the 0.001 level. In a sample of 248 teachers, the two test sessions were separated by an interval of one year. The test-retest reliabilities for the three subscales were: 0.60 for Emotional Exhaustion; 0.54 for Depersonalization; and 0.57 for Personal Accomplishment (Jackson, Schwab, and Schuler, 1986).

VALIDITY

Convergent validity

Convergent validity was demonstrated in several ways. First, an individual's MBI scores were correlated with behavioural ratings made independently by a person who knew the individual well (i.e. one's spouse or co-workers). Second, MBI scores were correlated with the presence of certain job characteristics that were expected to contribute to experienced burnout. Third, MBI scores were correlated with measures of various outcomes that had been hypothesized to be related to burnout. All three sets of correlations provided substantial evidence for the validity of the MBI.

External validation of personal experience

One type of validating evidence comes from outside observers whose independent assessments of an individual's experience corroborate the individual's self-rating. Within the job setting, a knowledgeable observer would be a person's co-worker. Accordingly, a group of 40 mental health workers were each asked to provide an anonymous behavioural evaluation of a designated co-worker who had also completed the MBI. The critical questions on this evaluation, in terms of validating the Emotional Exhaustion and Depersonalization subscales, were ratings of how "emotionally drained" the person was, and how he or she reacted to clients. As predicted, people who were rated by the co-worker as being emotionally drained by the job scored higher on Emotional Exhaustion ($r = 0.28$, $p < 0.28$) and on Depersonalization ($r = 0.56$, $p < 0.001$). Furthermore, people who were rated as appearing physically fatigued scored higher on Emotional Exhaustion ($r = 0.42$, $p < 0.01$) and on Depersonalization ($r = 0.50$, $p < 0.001$). It had been expected that high scores on Depersonalization would be reflected in the behaviour of frequent complaints about clients. Co-workers' ratings of this behaviour were indeed correlated with Depersonalization scores ($r = 0.32$, $p < 0.05$). The predicted correlation between co-worker ratings of the individual's satisfaction with the job and scores on Personal Accomplishment failed to achieve statistical significance.

Within the home setting, a knowledgeable observer would be the person's spouse, and so spouse evaluations were collected via a questionnaire survey of 142 policemen and their wives (Jackson and Maslach, 1982). The wives were asked to indicate the frequency of several of their husband's behaviours that were predicted to be reflective of the Emotional Exhaustion and Personal Accomplishment dimensions of the MBI (since the wives did not see their husbands working with people on the job, they were not asked to rate behaviours reflecting Depersonalization). Each wife's ratings were compared with her husband's MBI scores, and the resulting correlations were in line with the predictions. Thus, police who scored higher on Emotional Exhaustion were rated by their wives as coming home: upset and angry ($r = 0.34$, $p < 0.001$), tense or anxious ($r = 0.27$, $p < 0.001$), physically exhausted ($r = 0.20$, $p < 0.01$), and complaining about problems at work ($r = 0.26$, $p < 0.001$). Police who scored higher on Personal Accomplishment were rated by their wives as coming home in a cheerful or happy mood ($r = 0.25$, $p < 0.01$) and as doing work that was a source of pride and prestige for the family ($r = 0.24$, $p < 0.01$).

Dimensions of the job experience

The validity of the MBI is demonstrated further by data that confirm hypotheses about the relationships between various job characteristics and experienced burnout. Based on the findings of Maslach and Pines (1977), it was predicted that the greater the number of clients one must deal with, the higher the burnout scores on the MBI. Maslach and Jackson (1984) found precisely this pattern of response in a nation-wide survey of 845 public contact employees. When caseloads were very large (over 40 people served per day), scores were high on Emotional Exhaustion and Depersonalization and low on Personal Accomplishment. A study of 43 physicians in a California health maintenance organization (cited in Maslach and Jackson, 1982) found that those who spent all or most of their working time in direct contact with patients scored higher on Emotional Exhaustion ($r = 0.31$, $p < 0.05$). Emotional Exhaustion scores were lower for those physicians who spent some of their working hours in teaching ($r = -0.26$, $p < 0.05$) or administration ($r = -0.21$, $p < 0.10$). However, Personal

Accomplishment scores were unrelated to differences in the division of work duties.

A personal assessment of certain basic job dimensions is part of the Job Diagnostic Survey (JDS) (Hackman and Oldham, 1974, 1975). This measure and the MBI were completed by a sample of 91 social service and mental health workers. One dimension, "feedback from the job itself", measures the degree to which carrying out the work activities (i.e. contact with recipients) gives the employee direct and clear information about job performance. As had been predicted from earlier research (Pines and Kafry, 1978), higher scores on this job dimension were correlated with lower scores on Emotional Exhaustion (r = -0.24, $p < 0.05$) and on Depersonalization (r = -0.44, $p < 0.001$), and higher scores on Personal Accomplishment (r = 0.38, $p < 0.001$). "Dealing with others" assesses the degree to which the job requires the employee to work closely with people in carrying out the job activities. High scores on this dimension were weakly correlated with Emotional Exhaustion (r = 0.15, $p < 0.10$). On a third job dimension of "task significance," which assesses the degree to which the job has a substantial impact on the lives of other people, high scores were correlated positively with Personal Accomplishment (r = 0.19, $p < 0.05$), as had been predicted.

Personal outcomes

Additional validation of the MBI is provided by data that confirm hypothesized relationships between experienced burnout and various outcomes or personal reactions. Based on previous theorizing and research (Maslach, 1976), it was predicted that people experiencing burnout would be dissatisfied with opportunities for personal growth and development on the job. This hypothesis received support in a study of 180 nurses, social service and mental health workers. Scores on the JDS measure of "growth satisfaction" were negatively correlated with Emotional Exhaustion (r = -0.24, $p < 0.001$) and Depersonalization (r = -0.47, $p < 0.001$), and positively correlated with Personal Accomplishment (r = 0.41, $p < 0.001$). Previous work also suggested that burnout would be associated with the belief that one's work is not very meaningful or worthwhile. In support of this hypothesis, people scoring low on the JDS subscale of "experienced meaningfulness of the work (n = 91) scored higher on Depersonalization (r = -0.32, $p < 0.001$) and lower on Personal Accomplishment (r = 0.27, $p < 0.01$). Finally, the previous correlations of burnout with low feedback led to the prediction that employees who scored high on burnout would not know how effectively they were performing their job. And indeed, low scores on the JDS subscale of "knowledge of results" (n = 91) were correlated with high scores on Emotional Exhaustion (r = -0.31, $p < 0.01$) and Depersonalization (r = -0.31, $p < 0.01$), and with low scores on Personal Accomplishment (r = 0.20, $p < 0.05$).

Previous theorizing (Maslach, 1976) led to the prediction that burnout would be related to the desire to leave one's job. Support for this hypothesis found in the questionnaire survey of 142 police officers (Jackson and Maslach, 1982). The officer's MBI scores were highly predictive of intention to quit, R (6, 135) = 0.68, $p < 0.001$. Similarly, public contact workers who expressed an intention to leave theirjob within a year scored higher on Emotional Exhaustion ($r = .31, p < 0.001$) and Depersonalization ($r = .33, p < 0.001$), and lower on Personal Accomplishment ($r = -.21$, $p < 0.01$) (Maslach and Jackson, 1984).

Public contact employees who wished they could spend less time working with the public scored higher on Emotional Exhaustion ($r = .53$, $p < 0.001$) and Depersonalization ($r = .49$, $p < 0.001$), and lower on Personal Accomplishment ($r = -.43, p < .001$). The latter findings provide strong support for the hypothesis that people experiencing burnout would want to spend less time working with people.

Another hypothesized outcome of burnout is an impairment of one's relationships with people in general, both on and off the job (Maslach, 1976). In line with this prediction, physicians scoring high on Emotional Exhaustion (n = 43) were more likely to report that they wanted to get away from people ($r = 0.27$, $p < 0.05$). With respect to the co-workers themselves, staff scoring low on the JDS subscale of "peer and co-worker satisfaction" (n = 180) scored high on Emotional Exhaustion ($r = -0.16$, $p < 0.05$) and Depersonalization ($r = -0.41$, $p < 0.001$), and low on Personal Accomplishment ($r = 0.40$, $p < 0.001$).

The proposed relationship of burnout to difficulties with family and friends (Maslach, 1976) was tested in the study of 142 police officers and their wives (Jackson and Maslach, 1982). A police officer experiencing burnout was more likely to report that he gets angry at his wife or children (Depersonalization, $r = 0.16, p < 0.05$; Emotional Exhaustion, $r = 0.16$, $p < 0.05$). If he scored high on Emotional Exhaustion, he was also more likely to report that he wanted to be alone, rather than spend time with his family ($r = 0.16$, $p < 0.05$). He perceived his children as being more emotionally distant from him if he was experiencing Depersonalization ($r = 0.32, p < 0.001$) or feelings of low Personal Accomplishment ($r = —0.38$, $p < 0.001$). The officer scoring high on Depersonalization was also more likely to be absent from family celebrations ($r = 0.21, p < 0.01$). Reports of fewer friends were correlated with frequent feelings of Depersonalization ($r = 0.22, p < 0.01$). The officer's wife was also more likely to say that he and she did not share the same friends (Depersonalization, $r = 0.17$, $p < 0.05$).

Previous theorizing and research (Maslach, 1976) had suggested that burnout would be linked to such stress outcomes as insomnia and increased use of alcohol and drugs. Some supportive evidence is also provided by the study of police couples. As predicted, a police officer scoring high on Emotional Exhaustion was rated by his wife as having more frequent problems with insomnia ($r = 0.24$, $p < 0.01$). The officers themselves were more likely to report having a drink to cope with stress if they had high scores on Emotional Exhaustion ($r = 0.24$, $p < 0.01$) and to report taking tranquilizers when they scored low on Personal Accomplishment ($r = -0.18$, $p < 0.05$). This use of tranquilizers was corroborated by their wives, who were also more likely to report that their husbands used medications if they scored low on Personal Accomplishment ($r = -0.28$, $p < 0.01$) or high on Emotional Exhaustion ($r = 0.21, p < 0.05$).

Discriminant validity

Further evidence of the validity of the MBI was obtained by distinguishing it from measures of other psychological constructs that might be presumed to be confounded with burnout. For example, it is possible that the experience of burnout may be nothing more than the experience of dissatisfaction with one's job. Although one would expect the experience of burnout to have some relationship to lowered feelings of job satisfaction, it was predicted that they would not be so highly correlated as to suggest that they were actually the same thing. A comparison of subjects' scores on the MBI and the JDS measure of "general job satisfaction" ($n = 91$ social service and mental health workers) provides support for this reasoning. Job satisfaction had a moderate negative correlation with both Emotional Exhaustion ($r = -0.23$, $p < 0.05$) and Depersonalization ($r = -0.22$, $p < 0.05$), as well as a slightly positive correlation with Personal Accomplishment ($r = 0.17, p < 0.06$). However, since less than 6 per cent of the variance is accounted for by any one of these correlations, one can reject the notion that burnout is simply a synonym for job dissatisfaction.

It might also be argued that scores on the MBI are subject to distortion by a social desirability response set, since many of the items describe feelings that are contrary to professional ideals. To test this idea, a sample of 40 graduate students in social welfare was asked to complete both the MBI and the Crowne-Marlowe (1964) Social Desirability (SD) Scale. If reported burnout is not influenced by a social desirability response set, then the scores on the MBI and the SD Scale should be uncorrelated. Our results supported this hypothesis, since none of the MBI subscales were significantly correlated with the SD Scale at the 0.05 level.

DEMOGRAPHIC DATA

Group norms for various demographic variables are presented in Table 3. These norms are drawn from the 1986 MBI Manual and are based on a large number of research samples from both the United States and Canada (see Maslach and Jackson, 1986 for a more complete description of these samples). A review of these norms provides some indication of the relationship of certain demographic variables to the experience of burnout. However, these patterns must be interpreted with caution because many of the variables are clearly confounded with type of occupation, job status, job tenure, and so forth.

For example, one of the clearest trends is that burnout scores are highest for younger people and decline with increasing age. These results corroborate findings from earlier interviews (Maslach, 1976) that burnout is likely to occur within the first few years of one's career. If people have difficulty in coping effectively with burnout at this point, they may leave their profession entirely. Thus, the people in the older age range of our sample may be those who have survived the early stresses of their job and done well in their career. These findings are also in line with the positive

Table 3. Demographic norms for the MBI subscales

		MBI subscales					
		Emotional Exhaustion		Depersonal-ization		Personal Accomplishment	
Variable	N	M	SD	M	SD	M	SD
Sex							
Male	2247	19.86	10.47	7.43	5.99	36.29	6.76
Female	3421	20.99	10.66	7.02	6.34	36.50	6.56
Race							
Caucasian	1511	23.14	11.39	8.86	6.38	37.12	6.85
Black	199	17.80	10.53	5.19	5.01	37.62	7.10
Asian-Am.	32	25.03	13.61	10.58	7.96	34.75	7.78
Other	93	19.92	10.83	8.11	5.71	36.17	7.99
Age							
30 & under	1044	23.86	11.23	9.36	9.06	35.91	6.58
31-40	1197	22.26	10.78	8.25	6.05	37.26	6.63
41-50	641	20.24	11.06	6.66	5.56	38.19	7.67
51 & over	624	17.96	10.93	5.29	5.09	38.41	6.90
Marital status							
Single	732	24.28	11.19	9.35	6.43	35.89	6.71
Married	2017	19.95	10.63	7.04	6.02	38.04	6.74
Divorced	478	22.29	11.19	7.75	5.97	37.22	6.73
Other	281	23.01	12.36	7.34	6.43	37.30	7.01
Education							
No college	269	22.99	11.77	8.57	6.86	36.54	6.97
Some coll.	664	21.32	11.29	8.00	6.54	35.33	6.66
College	664	19.08	9.97	7.49	5.70	31.48	6.95
Postgrad.	1878	21.13	10.74	7.35	5.67	37.88	6.42

correlations between age and job satisfaction that are commonly reported (see Weaver, 1980).

There are no major sex differences in burnout. However, there is some relationship between marital status and burnout, with married employees showing lower burnout scores. More extensive research on these two variables and family status found that, compared to employees who had one or more children, childless employees had much higher scores on all three dimensions of burnout (Maslach and Jackson, 1985).

The overall relationship between burnout and level of education is difficult to interpret, given the fact that education is strongly confounded with job status. However, there seems to be a tendency for Depersonalization scores to decline with greater education. In addition, employees who have completed college but have not done postgraduate work tend to show greater burnout on one dimension (Personal Accomplishment) and less burnout on another (Emotional Exhaustion).

CONCLUSION

The development of the MBI was based on the need for an instrument to assess experienced burnout in a wide range of human service workers. Its inclusion in future research studies will allow us to achieve a better understanding of the personal, social and institutional variables that either promote or reduce the occurrence of burnout. In addition to the significance of this knowledge for theories of emotion and of job stress, such information will have the practical benefit of suggesting modifications in recruitment, training, and job design that may alleviate this serious problem.

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Author's address:

Christina Maslach, Department of Psychology, University of California, Berkeley, California 94720, U.S.A.